

Assisted birth

In the US, 4.8 percent of babies have an assisted birth. This is considered safe and can prevent further complications.

An assisted birth is a birth in which either forceps or a vacuum suction cup (also called a vacuum or ventouse extractor) are used to aid a vaginal delivery. There may be some mild side effects such as bruising after an assisted birth, but major complications are rare.

As with all medical interventions, the use of forceps or a vacuum extractor will only be offered when it is thought to be necessary for the health of the baby and/or mother.

The procedure

First, your doctor will discuss with you the need for the procedure and may also explain to you the potential complications. You may be asked to sign a consent form, although not all hospitals insist on this. Before the birth takes place, you will probably be given an episiotomy and local anesthetics or epidural anesthesia. Fresh covers may be placed over your legs and abdomen and a catheter will be inserted to empty your bladder. The doctor will then place the forceps around the baby's head or attach the vacuum cup and will encourage you to start pushing again when you have a contraction. Your baby should be born within around another 20 minutes from this time. There is less of a risk for a vaginal injury with a vacuum extraction than there is with forceps.

Forceps

Modern surgical forceps were invented by the British doctor Chamberlen in the seventeenth century and so have been in use for hundreds of years, although they have been modified over time. Forceps are an effective and reliable way to assist a delivery, but they have to be used with care by a properly trained doctor.

The forceps are placed so that they hold the sides of the baby's head near the ears and cheeks. The operator of the forceps then gently pulls the baby's head downward while you push during a contraction, and then will guide the baby out of the birth canal.

Forceps can also be used to adjust the baby's head before the birth if the baby is lying in an occiput posterior position, which is with his back facing the mother's back (see [Support in the first stage](#)), toward the end of labor. After turning the baby's head and body, delivery is then completed in the same way as described above.

Pros and cons of forceps There are several advantages of forceps over vacuum. First, they do not rely on any kind of machinery that could cause a problem during the birth. They work well even if your contractions are weak or if you are too exhausted to push. They have a low failure rate: if the doctor can get the forceps around your baby's head easily, he will usually be able to complete the vaginal birth of your baby without having to resort to a cesarean. The disadvantage of forceps is that you may be more likely to have vaginal or perineal damage than with vacuum.

Vacuum extractor

The vacuum, or vacuum extractor, was invented in the 1950s. It consists of a cup attached to a tube that connects to a suction pump. The cup, which can be hard or soft plastic or metal, is placed on top of the baby's head and held in place while the suction